BROWN UNIVERSITY HEALTH

DISCHARGE SUMMARY

Patient: Maria Santos (MRN: MRN-900001)
Date of Admission: November 10, 2024
Date of Discharge: November 14, 2024
Attending Physician: Dr. Amy Chen, Cardiology

PRINCIPAL DIAGNOSIS

Acute decompensated heart failure (ICD-10: I50.21)

SECONDARY DIAGNOSES

- Type 2 diabetes mellitus (E11.9)
- Essential hypertension (I10)
- Chronic kidney disease, stage 3 (N18.3)

HOSPITAL COURSE

Mrs. Santos presented to the Emergency Department with progressive dyspnea and bilateral lower extremity edema over 5 days. Initial BNP was 1,240 pg/mL. Chest X-ray showed bilateral pleural effusions and pulmonary vascular congestion. She was admitted to the cardiac care unit and started on IV furosemide with strict I&O monitoring.

Over the course of her admission, she diuresed 4.2 liters with significant improvement in symptoms. Echocardiogram showed LVEF of 35%, reduced from 45% on prior study six months ago. Renal function remained stable with creatinine 1.4 mg/dL.

MEDICATIONS AT DISCHARGE

1. Furosemide 40mg PO twice daily (increased from 20mg daily)
2. Lisinopril 10mg PO daily (continued)
3. Carvedilol 12.5mg PO twice daily (new)
4. Metformin 500mg PO twice daily (continued)
5. Potassium chloride 20mEq PO daily (new, with diuretic)

FOLLOW-UP PLAN

- Cardiology follow-up with Dr. Chen in 7 days
- Daily weights; call if weight gain >2 lbs in 24 hours
- Repeat echocardiogram in 3 months
- Lab work (BMP, BNP) in 1 week
- Heart failure education completed with nursing staff